

Uma Clinic

14, Bhavani Complex , Kamrej Char Rasta, Surat - 394 185

Ph: -9408654092

Prescription

NAME : URMILA RITEN BHAI HIRPARA

NO. : 64902

ADDRESS : SHANTI NAGAR

PATIENT ID : 22916

AGE & SEX : 45/FEMALE

DATE : 08/04/2024

TREATING DR. : BAKUL PATEL

SR	TYPE	MEDICINE NAME	DOSE	TIMES/DAY	DURATION	QTY	INSTRUCTION
1		TAB. MBSON SL		1-0-0	30 Days	30	
2		Tab. Caldison D3		1-0-0	4 Days	4	

કરી જતાવવા માટે **30** દિવસ પછી આવવું

Name : URMILABEN HIRAPARA

Ref By : UMA CLINIC

Age/Sex : 45 Yrs./F

Date : 04/04/2024

Report ID. : 1



9909692605

HAEMOGRAM PROFILE

TEST	RESULT	UNIT	REFERENCE INTERVAL
BLOOD COUNTS & INDICES			
Haemoglobin	↓ : 11.00	gm%	12.0 - 16.0 gm%
Total RBC	↓ : 3.90	mill/cmm	4.2 - 5.4 mill/cmm
PCV	↓ : 36.90	%	37 - 47 %
MCV	: 94.62	fL	80 - 96 fL
MCH	: 28.21	pg	27 - 31 pg
MCHC	↓ : 29.81	%	32 - 36 %
RDW	: 15.70	%	10.00 - 16.00 %
Total WBC	: 4700	/cmm	4,000 - 10,000/cmm
Platelet Count	: 388,000	/cmm	1.5 - 4.0 Lac/cmm.
DIFFERENTIAL LEUCOCYTES COUNT			
Band Cells	: 00	%	00 - 06 %
Neutrophils	↓ : 46	%	55 - 70 %
Lymphocytes	↑ : 43	%	20 - 40 %
Eosinophils	: 04	%	01 - 06 %
Monocytes	: 07	%	02 - 08 %
Basophils	: 00	%	00 - 01 %
Platelet In Smear	: ADEQUATE		
PARASITES	: M.P. NOT SEEN		
(By Thin & Thick Smear)			
Blood Group	: " O "		
Rh Factor	: " POSITIVE "		
(Anti D.)			

(By Automated Cell Counter ERMA PCE 210, Japan)

Kalpesh Giri

M.Sc. (Micro)
PG.DMLT

+91 98250 55940

Anand Joshi

B.Sc. (Micro)
PG.DMLT

+91 98252 90010

86 86 8 86 4 86

Sign.

TEST REPORT SUBJECT TO TECHNICAL LIMITATION AND SHOULD BE CLINICALLY RELATED, LABORATORY MAY BE CONTACTED WHENEVER REQUIRED.

A/140-141, Bhavani Complex, Beside Kanchan Hospital, Kamrej Char Rasta, Ta. Kamrej, Dist. SURAT-394 185.

M. 098254 51433 Ph. (02621) 251433

e-mail : info@omclinicallab.com | website : www.omclinicallab.com

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BLOOD GLUCOSE ANALYSIS

<u>TEST</u>	<u>RESULT</u>	<u>UNIT</u>	<u>REFERENCE INTERVAL</u>
<u>FASTING</u>			
Blood Glucose	: 76.0	mg/dL	70 - 110 mg/dL
Urine Glucose	: NIL		
Urine Acetone	: NIL		

ARTHRITIS PROFILE**C. REACTIVE PROTEIN (CRP)**

Normal	: < 6.0.mg/L	mg/L
Concentration	: 1.4	mg/L
Technique	: IMMUNOTURBIDOMETRE	

RHEUMATOID ARTHRITIS FACTOR (RA)

Concentration	: 7.7	IU/ML	< 20.00 IU/ML
Technique	: IMMUNOTURBIDOMETRE		

BIOCHEMISTRY

Serum Amylase	↑ : 151.00	U/L	00-90 U/L
---------------	-------------------	-----	-----------

Done on MICRO LAB RX - 50 Analyser.

Kalpesh GiriM.Sc (Micro)
PG DMLT

+91 98250 55940

Anand JoshiB.Sc (Micro)
PG DMLT

+91 98252 90010


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CARDIAC RISK MARKERS

<u>TEST</u>	<u>RESULT</u>	<u>UNIT</u>	<u>REFERENCE INTERVAL</u>
High Sensitivity CRP	: 0.100	mg/L	Low Risk : < 1 mg/L Avg. Risk : 1 - 3 mg/L High Risk : > 3 mg/L

Kalpesh Giri

M.Sc. (Micro)
PG. DMAT

+91 98250 55940

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EXAMINATION OF BLOOD FOR HIVTESTResult
MethodRESULT: TEST IS NON-REACTIVE
: TRI DOT

LEGEND :

1. Positive initial screening test for HIV antibody should be reconfirmed by ELISA test from another sample of the same patient collected after an interval of few days using ELISA test kit from another manufacturer.
2. Final confirmation should be done by performing Western blot test.
3. A negative test does not exclude the possibility of prior exposure especially a recent one.
4. Final diagnosis of HIV infection must be based on clinical correlation with laboratory test result.

Kalpesh Giri

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PG.DMLT

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LIVER FUNCTION TESTTESTRESULTUNITREFERENCE INTERVAL**S. BILIRUBIN**

Bilirubin - Total

: 0.30 mg/dL

0.0 - 1.0 mg/dL

Bilirubin - Direct

: 0.10 mg/dL

Upto 0.3 mg/dL

Bilirubin - Indirect

: 0.20 mg/dL

0.1 - 1.0 mg/dL

SGPT

: 16 IU/L

10 - 40 IU/L

SGOT

: 23 IU/L

Upto 40 IU/L

Serum Alkaline Phosphatase

: 38 U/L

37 - 147 U/L

SERUM PROTEIN

Serum Protein Total

: 7.10 g/dL

6.6 - 8.3 g/dL

Albumin

: 4.00 g/dL

3.5 - 5.0 g/dL

Globulin

: 3.10 g/dL

2.3 - 3.5 g/dL

A/G Ratio

↓ : **1.3** : 1

1.5 - 2.5 : 1

Serum Australia Antigen-HBsAg

: TEST IS NEGATIVE

By Chromatographic Immunoassay

Kit Used

: CDR HEPACHEK

TEST DONE ON HEMOSTAR XF 1.0, HAEMOSTASIS ANALYZER

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RENAL FUNCTION TEST

TEST	RESULT	UNIT	REFERENCE INTERVAL
Blood Urea	: 34.00	mg/dL	10.00 - 45.00 mg/dL
Blood Urea Nitrogen - BUN	: 15.90	mg/dL	4.50 - 19.00 mg/dL
Serum Creatinine	: 1.00	mg/dL	0.6 - 1.4 mg/dL
Serum Protein Total	: 7.10	g/dL	6.6 - 8.3 g/dL
Serum Albumin	: 4.00	g/dL	3.5 - 5.0 g/dL
Serum Globulin	: 3.10	g/dL	2.3 - 3.5 g/dL
A/G Ratio	↓ : 1.29	: 1	1.5 - 2.5 : 1
Serum Sodium (Na)	: 140.00	mEq/L	135 - 145 mEq/L
Serum Potassium (K)	: 3.80	mEq/L	3.5 - 5.5 mEq/L
Serum Chloride (Cl)	↑ : 107.00	mEq/L	96 - 106 mEq/L
Serum Uric Acid	: 2.80	mg/dL	2.4 - 5.7 mg/dL
Serum Calcium	: 8.60	mg/dL	8.5 - 10.5 mg/dL

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REPORT OF BLOOD EXAMINATION

TEST	RESULT	UNIT	REFERENCE INTERVAL
Serum Cholesterol	: 164.0	mg/dL	80 - 200 mg/dL
Serum Triglyceride	: 47.0	mg/dL	00 - 170 mg/dL
HDL Cholesterol	: 58.0	mg/dL	35 - 70 mg/dL
LDL Cholesterol	: 96.6	mg/dL	Upto 150 mg/dL
CHOL./HDL Chol. Ratio	: 2.83	: 1	Less than 5
LDL Chol/HDL Chol Ratio	: 1.67	: 1	Upto 5.00 : 1
VLDL	: 9.40	mg/dL	Upto 30 mg/dL

IDEAL CHOLESTEROL VALUES FOR INDIAN PEOPLE

* For Healthy People :

Serum Cholesterol	Less than 200 mg/dL
Serum LDL	Less than 100 mg/dL
Serum HDL	More than 45 mg/dL
Serum Triglyceride	Less than 150 mg/dL
Serum VLDL	Less than 24 mg/dL

* For Heart, Diabetes, High B.P. Patients:

Serum Cholesterol	Less than 160 mg/dL
Serum LDL	Less than 80 mg/dL
Serum HDL	More than 50 mg/dL
Serum Triglyceride	Less than 140 mg/dL
Serum VLDL	Less than 24 mg/dL

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REPORT OF BLOOD EXAMINATION

TEST	RESULT	UNIT	METHOD	REFERENCE INTERVAL
Hb A1C - Glycated Hb Glycated Haemoglobin	: 5.7	%	BORONET AFFINITY	Non Diabetic : 4.3 - 6.3 % Good Control : 6.4 - 7.5 % Moderate Control : 7.5 - 9.0 % Poor Control : 9.0 % & Above
Estimated Average Glucose eAG	: 117	mg%		

Notes :

Criteria for the diagnosis of diabetes

1. HbA1c $\geq$ 6.5*
Or
 2. Fasting plasma glucose $>$ 126 mg/dl. Fasting is defined as no caloric intake at least for 8 hrs.
Or
 3. Two hour plasma glucose $\geq$ 200 mg/dl during an oral glucose tolerance test by using a glucose load containing equivalent of 75 gm anhydrous glucose dissolved in water
Or
 4. In a patient with classic symptoms of hyperglycemic crisis, a random plasma glucose $\geq$ 200 mg/dl.
- *In the absence of unequivocal hyperglycemia, criteria 1-3 should be confirmed by repeat testing. American diabetes association. Standards of medical care in diabetes 2011. Diabetes care 2011;34:S11.

Limitation of HbA1c

1. In patients with Hb variants even analytically correct results do not reflect the same level of glycemic control that would be expected in patients with normal population.
2. Any cause of shortened erythrocyte survival or decreased mean erythrocyte survival or decreased mean erythrocyte age e.g. hemolytic diseases, pregnancy, significant recent/chronic blood loss etc, will reduce exposure of RBC to glucose with consequent decrease in HbA1c values.
3. glycated HbF is not detected by this assay and hence specimens containing high HbF ($>$ 10 %) may result in lower HbA1c values than expected.

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EXAMINATION OF URINE

TEST	RESULT	UNIT
Sample	: FASTING	
<u>PHYSICAL EXAMINATION</u>		
Quantity	: 20 ML	mL
Colour	: YELLOW	
Transparency	: S.TURBID	
Specific Gravity	: Q.N.S	
Reaction	: ACIDIC	
<u>CHEMICAL EXAMINATION</u>		
Albumin	: ABSENT	
Sugar	: ABSENT	
Acetone	: ABSENT	
Bile Salts	: ABSENT	
Bile Pigments	: ABSENT	
<u>MICROSCOPIC EXAMINATION</u>		
Pus Cells / hpf	: 1-2	
RBC / hpf	⚡ : 5-6	
Epithelial Cells/ hpf	: 3-4	
Crystals / hpf	: ABSENT	
Amorphous Phosphate	: ABSENT	
Cast	: ABSENT	
Mucus	: ABSENT	

Kalpesh Giri
M.Sc. (Micro)
PG.DMLT
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LABORATORY REPORT

Name : Mrs URMILABEN HIRAPARA	Sex/Age : Female / 45 Years	Case ID : 40403603170
Ref. By :	Dis. At :	Pt. ID : 3488241
Bill. Loc. : OM PATHOLOGY LABORATORY KAMREJ SURAT		Pt. Loc. :
Reg Date and Time : 04-Apr-2024 13:38	Sample Type : Serum	Mobile No. :
Sample Date and Time : 04-Apr-2024 13:38	Sample Coll. By : non NAL	Ref Id1 :
Report Date and Time : 04-Apr-2024 16:08	Acc. Remarks :	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
Thyroid Function Test				
Triiodothyronine (T3) CMIA	104.87	ng/dL	70 - 204	
Thyroxine (T4) CMIA	7.72	µg/dL	5.5 - 11.0	
TSH CMIA	1.847	µIU/mL	0.4 - 4.94	

INTERPRETATIONS

- Circulating TSH measurement has been used for screening for euthyroidism, screening and diagnosis for hyperthyroidism & hypothyroidism. Suppressed TSH ($<0.01 \mu\text{IU/mL}$) suggests a diagnosis of hyperthyroidism and elevated concentration ($>7 \mu\text{IU/mL}$) suggest hypothyroidism. TSH levels may be affected by acute illness and several medications including dopamine and glucocorticoids. Decreased (low or undetectable) in Graves disease. Increased in TSH secreting pituitary adenoma (secondary hyperthyroidism), PRTN and in hypothalamic disease thyrotropin (tertiary hyperthyroidism). Elevated in hypothyroidism (along with decreased T4) except for pituitary & hypothalamic disease.
- Mild to modest elevations in patient with normal T3 & T4 levels indicates impaired thyroid hormone reserves & incipient hypothyroidism (subclinical hypothyroidism).
- Mild to modest decrease with normal T3 & T4 indicates subclinical hyperthyroidism.
- Degree of TSH suppression does not reflect the severity of hyperthyroidism, therefore, measurement of free thyroid hormone levels is required in patient with a suppressed TSH level.

CAUTIONS

Sick, hospitalized patients may have falsely low or transiently elevated thyroid stimulating hormone. Some patients who have been exposed to animal antigens, either in the environment or as part of treatment or imaging procedure, may have circulating antianimal antibodies present. These antibodies may interfere with the assay reagents to produce unreliable results.

TSH ref range in pregnancy

First trimester
Second trimester
Third trimester

Reference range (microIU/ml)

0.24 - 2.00
0.43-2.2
0.8-2.5

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

(N)

Dr. Prashant Naik

M.D.(Path),D.C.P.

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Test reports are subject to technical limitations and should be clinically co-related. Laboratory may be contacted whenever required.

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Neuberg Abha Laboratory Private Limited

(Previously known as Abha Laboratory Pvt Ltd)

Regd. Office : 2nd Floor, Rajratna Chambers,
Dabgarwad, Zapata Showroom Lane,
Nr. Bhagal, Surat - 395003
Ph.: 0261-2425253 / 9099945480

Shankheswar Complex,
Opp. Raymond Showroom, Nr. Torrent Power
Majuragate, Surat - 395001
Ph.: 0261-2465061, 62, 63 / 9909946480

Aadi Vatsalya Building 8-A,
Opp. Old RTO Office, Ring Road,
Athwa Gate, Surat - 395002
Mob.: 3500500/7211126999

Email : info@neubergabha.com | Website : www.neubergabha.com | CIN : U74999GJ2020PT



LABORATORY REPORT

Name : Mrs URMILABEN HIRAPARA	Sex/Age : Female / 45 Years	Case ID : 40403603170
Ref. By :	Dis. At :	Pt. ID : 3488241
Bill. Loc. : OM PATHOLOGY LABORATORY KAMREJ SURAT		Pt. Loc. :
Reg Date and Time : 04-Apr-2024 13:38	Sample Type : Serum	Mobile No. :
Sample Date and Time : 04-Apr-2024 13:38	Sample Coll. By : non NAL	Ref Id1 :
Report Date and Time : 04-Apr-2024 16:09	Acc. Remarks :	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
VITAMIN B - 12				
Vitamin B - 12 Level CMIA	240.0	pg/mL	211 - 911	

Introduction :

Vitamin B12, a member of the corrin family, is a cofactor for the formation of myelin, and along with folate, is required for DNA synthesis. Levels above 300 or 400 are rarely associated with B12 deficiency induced hematological or neurological disease.

Clinical Significance :

Causes of Vitamin B12 deficiency can be divided into three classes: Nutritional, malabsorption syndromes and gastrointestinal causes. B12 deficiency can cause Megaloblastic anemia (MA), nerve damage and degeneration of the spinal cord. Lack of B12 even mild deficiencies damages the myelin sheath. The nerve damage caused by a lack of B12 may become permanently debilitating. The relationship between B12 and MA is not always clear that some patients with MA will have normal B12 levels; conversely, many individuals with B12 deficiency are not afflicted with MA.

Decreased in:

Iron deficiency, normal near-term pregnancy, vegetarianism, partial gastrectomy/ileal damage, celiac disease, use of oral contraception, parasitic competition, pancreatic deficiency, treated epilepsy and advancing age.

Increased in:

Renal failure, liver disease and myeloproliferative diseases.
Variations due to age Increases: with age.
Temporarily Increased after Drug.
Falsely high in Deteriorated sample.

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

(Signature)

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Reg Date and Time : 04-Apr-2024 13:38	Sample Type : Serum	Mobile No. :
Sample Date and Time : 04-Apr-2024 13:38	Sample Coll. By : non NAL	Ref Id1 :
Report Date and Time : 04-Apr-2024 16:03	Acc. Remarks :	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
25 OH Cholecalciferol (D2+D3) CMIA	L 6.10	ng/mL	Adult: 30 - 100 Normal Level 20 - <30 Insufficiency < 20 Deficiency > 150 Toxicity Pediatric 20 - 100 Normal Level 15 - <20 Insufficiency < 15 Deficiency	

25-OH-VitD plays a primary role in the maintenance of calcium homeostasis. It promotes intestinal calcium absorption and, in concert with PTH, skeletal calcium deposition, or less commonly, calcium mobilization. Modest 25-OH-VitD deficiency is common; in institutionalised elderly, its prevalence may be >50%. Although much less common, severe deficiency is not rare either. Reasons for suboptimal 25-OH-VitD levels include lack of sunshine exposure, a particular problem in Northern latitudes during winter; inadequate intake; malabsorption (e.g. due to Celiac disease); depressed hepatic vitamin D 25-hydroxylase activity, secondary to advanced liver disease; and enzyme-inducing drugs, in particular many antiepileptic drugs, including phenytoin, phenobarbital, and carbamazepine, that increase 25-OH-VitD metabolism. Hypervitaminosis D is rare, and is only seen after prolonged exposure to extremely high doses of vitamin D. When it occurs, it can result in severe hypercalcemia and hyperphosphatemia.

INTERPRETATION

- Levels <10 ng/mL may be associated with more severe abnormalities and can lead to inadequate mineralization of newly formed osteoid, resulting in rickets in children and osteomalacia in adults. In these individuals, serum calcium levels may be marginally low, and parathyroid hormone (PTH) and serum alkaline phosphatase are usually elevated. Definitive diagnosis rests on the typical radiographic findings or bone biopsy/histomorphometry.
- Patients who present with hypercalcemia, hyperphosphatemia, and low PTH may suffer either from ectopic, unregulated conversion of 25-OH-VitD to 1,25 (OH)₂-VitD, as can occur in granulomatous diseases, particularly sarcoidosis, or from nutritionally-induced hypervitaminosis D. Serum 1,25 (OH)₂-VitD levels will be high in both groups, but only patients with hypervitaminosis D will have serum 25-OH-VitD concentrations of >80 ng/mL, typically >150 ng/mL.
- Patients with CKD have an exceptionally high rate of severe vitamin D deficiency that is further exacerbated by the reduced ability to convert 25-OH-VitD into the active form, 1,25 (OH)₂-VitD. Emerging evidence also suggests that the progression of CKD & many of the cardiovascular complications may be linked to hypovitaminosis D.
- Approximately half of Stage 2 and 3 CKD patients are nutritional vitamin D deficient (25-OH-VitD, less than 30 ng/mL), and this deficiency is more common among stage 4 CKD patients. Additionally, calcitriol (1,25 (OH)₂-VitD) levels are also overtly low (less than 22 pg/mL) in CKD patients. Similarly, vast majority of dialysis patients are found to be deficient in nutritional vitamin D and have low calcitriol levels. Recent data suggest an elevated PTH is a poor indicator of deficiencies of nutritional vitamin D and calcitriol in CKD patients. CAUTIONS Long term use of anticonvulsant medications may result in vitamin D deficiency that could lead to bone disease; the anticonvulsants most implicated are phenytoin, phenobarbital, carbamazepine, and valproic acid.

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

(Signature)

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Regd. Office : 2nd Floor, Rajratna Chambers,
Dabgarwad, Zapata Showroom Lane,
Nr. Bhagal, Surat - 395003
Ph.: 0261-2425253 / 9099945480

Shankheswar Complex,
Opp. Raymond Showroom, Nr. Torrent Power
Majuragate, Surat - 395001
Ph.: 0261-2465061, 62, 63 / 9909946480

Aadi Vatsalya Building 8-A,
Opp. Old RTO Office, Ring Road,
Athwa Gate, Surat - 395002
Mob.: 3500500/7211126999

Email : info@neubergabha.com | Website : www.neubergabha.com | CIN : U74999GJ2020PT



LABORATORY REPORT



Name : Mrs URMILABEN HIRAPARA	Sex/Age : Female / 45 Years	Case ID : 40403603170
Ref. By :	Dis. At :	Pt. ID : 3488241
Bill. Loc. : OM PATHOLOGY LABORATORY KAMREJ SURAT		Pt. Loc. :
Reg Date and Time : 04-Apr-2024 13:38	Sample Type : Serum	Mobile No. :
Sample Date and Time : 04-Apr-2024 13:38	Sample Coll. By : non NAL	Ref Id1 :
Report Date and Time : 04-Apr-2024 16:03	Acc. Remarks :	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
25 OH Cholecalciferol (D2+D3) CMIA	L 6.10	ng/mL	Adult: 30 - 100 Normal Level 20 - <30 Insufficiency < 20 Deficiency > 150 Toxicity Pediatric 20 - 100 Normal Level 15 - <20 Insufficiency < 15 Deficiency	

25-OH-VitD plays a primary role in the maintenance of calcium homeostasis. It promotes intestinal calcium absorption and, in concert with PTH, skeletal calcium deposition, or less commonly, calcium mobilization. Modest 25-OH-VitD deficiency is common; in institutionalised elderly, its prevalence may be >50%. Although much less common, severe deficiency is not rare either. Reasons for suboptimal 25-OH-VitD levels include lack of sunshine exposure, a particular problem in Northern latitudes during winter; inadequate intake; malabsorption (e.g. due to Celiac disease); depressed hepatic vitamin D 25-hydroxylase activity, secondary to advanced liver disease; and enzyme-inducing drugs, in particular many antiepileptic drugs, including phenytoin, phenobarbital, and carbamazepine, that increase 25-OH-VitD metabolism. Hypervitaminosis D is rare, and is only seen after prolonged exposure to extremely high doses of vitamin D. When it occurs, it can result in severe hypercalcemia and hyperphosphatemia.

INTERPRETATION

- Levels <10 ng/mL may be associated with more severe abnormalities and can lead to inadequate mineralization of newly formed osteoid, resulting in rickets in children and osteomalacia in adults. In these individuals, serum calcium levels may be marginally low, and parathyroid hormone (PTH) and serum alkaline phosphatase are usually elevated. Definitive diagnosis rests on the typical radiographic findings or bone biopsy/histomorphometry.
- Patients who present with hypercalcemia, hyperphosphatemia, and low PTH may suffer either from ectopic, unregulated conversion of 25-OH-VitD to 1,25 (OH)₂-VitD, as can occur in granulomatous diseases, particularly sarcoidosis, or from nutritionally-induced hypervitaminosis D. Serum 1,25 (OH)₂-VitD levels will be high in both groups, but only patients with hypervitaminosis D will have serum 25-OH-VitD concentrations of >80 ng/mL, typically >150 ng/mL.
- Patients with CKD have an exceptionally high rate of severe vitamin D deficiency that is further exacerbated by the reduced ability to convert 25-OH-VitD into the active form, 1,25 (OH)₂-VitD. Emerging evidence also suggests that the progression of CKD & many of the cardiovascular complications may be linked to hypovitaminosis D.
- Approximately half of Stage 2 and 3 CKD patients are nutritional vitamin D deficient (25-OH-VitD, less than 30 ng/mL), and this deficiency is more common among stage 4 CKD patients. Additionally, calcitriol (1,25 (OH)₂-VitD) levels are also overtly low (less than 22 pg/mL) in CKD patients. Similarly, vast majority of dialysis patients are found to be deficient in nutritional vitamin D and have low calcitriol levels. Recent data suggest an elevated PTH is a poor indicator of deficiencies of nutritional vitamin D and calcitriol in CKD patients. CAUTIONS Long term use of anticonvulsant medications may result in vitamin D deficiency that could lead to bone disease; the anticonvulsants most implicated are phenytoin, phenobarbital, carbamazepine, and valproic acid.

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

(Signature)

Dr. Prashant Naik
M.D.(Path),D.C.P.

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Test reports are subject to technical limitations and should be clinically co-related. Laboratory may be contacted whenever required.

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